Machine Translated by Google

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°177/MVE-BDBV/07/11/2026

Reporting date: November 7, 2026 Publication date: November 8, 2026

HIGHLIGHTS (last 24 hours)

Over the past 24 hours, forty-one (41) new confirmed cases and 21 confirmed deaths, including 12 community deaths and 9 deaths within treatment centers, have been reported. The new cases were recorded in Ituri (24), North Kivu (12), and Haut-Uélé (5). A total of 423 (To the team: Q2) new samples were analyzed in the laboratories that submitted their data; 41 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 9.7%. No new health zones were affected in the past 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10920	5323	48.7%	1010	2,940	69.8%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

On November 7, 2026, forty-one (41) new confirmed cases of Ebola virus disease (EVD) were reported in three provinces: Ituri (24 cases), North Kivu (12 cases), and Haut-Uélé (5 cases), representing a 42.3% decrease compared to the previous day. Among these cases, 12 community deaths were reported (5 in Ituri, 6 in North Kivu, and 1 in Haut-Uélé). In addition, 9 confirmed deaths were recorded in Ebola Treatment Centers (ETCs) (5 in Ituri, 3 in North Kivu, and 1 in Haut-Uélé), bringing the total number of confirmed deaths reported during this period to 21.

In addition, 5 patients (1 in North Kivu, 1 in Tshopo and 3 in Ituri) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded 10,920 confirmed cases, including 5,323 deaths, representing an overall case fatality rate of 48.7%. Ituri remains the epicenter of the epidemic, accounting for 69.7% of cumulative confirmed cases and 58.5% of new reported cases, while North Kivu represents 29.3% of new cases and has a case fatality rate of 54.8%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic, namely: Ituri, North Kivu, Haut-Uélé, Tshopo, South Kivu, Bas-Uélé, South Ubangi and North Ubangi.

Machine Translated by Google

Table 1. Distribution of confirmed cases and deaths by affected province, as of November 7, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	24	7,614	3,594	47.2%	28/36 (77.8%)
North Kivu	12	2,697	1,478	54.8%	17/34 (50.0%)
Haut-Uélé	5	486	213	43.8%	7/13 (53.8%)
Tshopo	0	96	31	32.3%	7/23 (30.4%)
South Kivu	0		1	33.3%	1/34 (2.9%)
Bas-Uélé	0	3	4	30.8%	4/11 (36.4%)
South Ubangi	0		1	50.0%	1/16 (6.3%)
North Ubangi	0	13 2 9	1	11.1%	2/11 (18.2%)
Total	**41**	**10,920**	**5,323**	**48.7%**	**67/178 (37.6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of November 7, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cases (n)	Deaths (n)	Case fatality rate	New Case	Death community deaths (Swab+)	Confirmed intra-CTE	Total confirmed deaths
Ituri	7614	3594	47.2%	24	5	5	10
Adja	11	1	9.1%				0
Aru	25	7	28.0%				0
Ariwara		2	25.0%				0
Aungba	8	5	41.7%				0
Bamboo	12 220	50	22.7%				0
Boga	2	2	100.0%				0
Bunia	2041	717	35.1%	3	0	0	0
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	85	31	36.5%	1	0	0	0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	40	13	32.5%				0
Komanda 193		119	61.7%				0
Lita	338	169	50.0%			0	1
Logo	14	10	71.4%	30	10	1	1
Lolwa	37	11	29.7%				0
Mahagi	6	1	16.7%				0
Mambasa 132		68	51.5%	1	1	0	1
Mandima 550		292	53.1%	8	2	0	2
Mangala	415	177	42.7%	1	0	0	0
Mongbwalu 731		360	49.2%	0	0	2	2
Nia-Nia 344		201	58.4%				0
Nizi 866		318	36.7%	4	0	1	1
Nyankunde 141		45	31.9%				0
Rimba	29	7	24.1%				0

Machine Translated by Google

Rwampara 1219		444	36.4%	3	1	1	2
Tchomia 86		27	31.4%				0
To ventilate	0	493	N / A				0
North Kivu 2697		1478	54.8%	12	6	3	9
Blessed	815	525	64.4%	6	4	0	4
Good			40.7%				0
Butembo			58.5%	2	0	0	0
Goma	27,349 1	11,204,0	0.0%				0
Kalunguta 110		47	42.7%	1	1	2	3
Katwa 684		423	61.8%				0
Kayna	3	2	66.7%				0
Kyondo	47	22	46.8%				0
Lubero	43	16	37.2%				0
Mabalako 127		41	32.3%	3	1	0	1
Alimbongo 6		4	66.7%				0
Manguredjipa 16		5	31.3%				0
Masereka 48		15	31.3%				0
Musienene 164		81	49.4%				0
Mutwanga 95		22	23.2%				0
Oicha 104		40	38.5%				0
Vuhovi 58		20	34.5%				0
Haut-Uélé 486		213	43.8%	5	1	1	2
Boma	46	20	43.5%				0
Mangbetu							
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	121	57	47.1%	0	0	1	1
Pawa	126	68	54.0%	4	1	0	1
Rungu	9	5	55.6%				0
Wamba 180		61	33.9%		0	0	0
Tshopo 96		31	32.3%	10	0	0	0
Bafwasende 18		3	16.7%				0
Kabondo	24	5	20.8%				0
Lubunga	1	0	0.0%				0
Makiso-	37	15	40.5%				0
Kisangani							
Mangobo 10		7	70.0%				0
Tshopo	5	0	0.0%				0
Wanie-	1	1	100.0%				0
Rukula							
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa 3		1	33.3%				0
Bas Uélé	13	4	30.8%	0	0	0	0
Buta			100.0%				0
Viadana	1 6		16.7%				0
Dakwa	1	1 1 0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi 2 2		1	50.0%	0	0	0	0
Bulu		1	50.0%				0
North Ubangi 9		1	11.1%	0	0	0	0
Wapinda 5		1	20.0%				0
Yakoma 4		0	0.0%				0
Total 5323	10920		48.7%	41	12	9	21

***These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.**

NA: Not Applicable.

Machine Translated by Google

Table 3. Status of alerts notified by province for November 7, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	113 3 86 4 1		116	23	2	90	1	25	23
Bas Uélé	250 70		90	1	3	85			0
Ituri			1320 106 2095		69	1 144	1 1	4	90
North Kivu 2007 38 0			152 14		87	1 855	1	179	118
South Ubangi	14			3	0	11	0	239 3	0
North Ubangi ND 49		ND	ND	ND	ND	ND	ND	ND	ND
Tshopo		2	51	9	2	40	0	11	1
Total General	**3,519,167 3,686,294**				**163 3 225**		**4**	**461**	**232**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

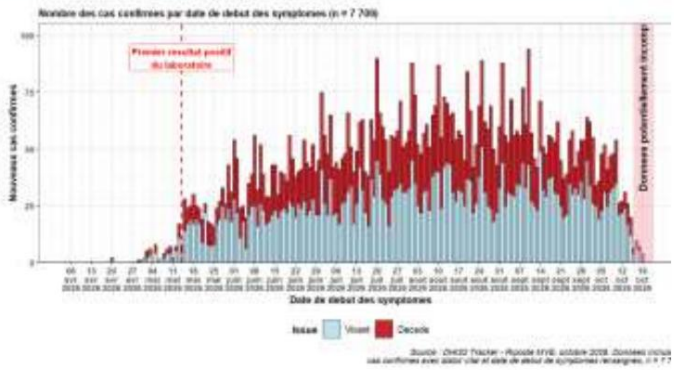
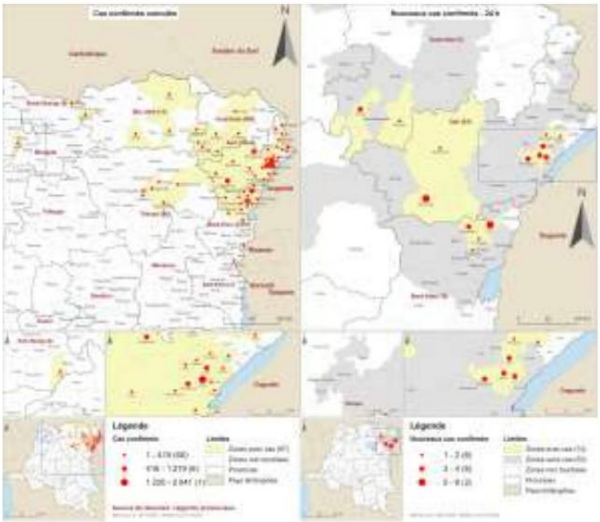


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the provinces **2.**
RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.2. Main Actions

As of November 7, 2026, 3,686 alerts had been reported in the six provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, Sud-Ubangi, and Tshopo), including 3,519 alerts of living persons and 167 of deaths; 3,686 were verified (100%). Of these, 457 (12.4%) were validated as

Machine Translated by Google

of the 461 suspected cases investigated, 232 were transferred to CT/CTE (Table 3).

As of November 7, 2026, a total of 33,346 contacts were under follow-up in the five provinces that reported contacts to follow up, of which 23,276 were seen in the last 24 hours, representing a follow-up rate of 69.8%, below the required threshold of 95%.

Table 4. Status of contact tracing by province, as of November 7, 2026

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	10,416	8,630	82.9%
Haut-Uélé	2,250	1821	80.9%
North Kivu	19,138	11,931	62.3%
Tshopo	1,275	628	49.3%
Bas-Uélé	267	266	99.6%
South Ubangi	ND	ND	ND
North Ubangi	0	0	N / A
Total	33,346	23,276	69.8%

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For November 6, 2026, four (4) alerts were notified, two (2) in Ituri and two (2) in North Kivu, with none reported in South Kivu, Tshopo, or Bas-Uélé (Table 5). In Ituri, the two live alerts were validated and transferred. In North Kivu, at the Kasitu PoC (Beni), one live alert was validated and referred to the Beni General Referral Hospital (100%; 1/1), and the deceased body from Lukaya (Ituri) was intercepted, validated, and sent to the morgue for swab (100%; 1/1). In total, the 3 validated live alerts were transferred to CT/CTE (100%; 3/3), the intercepted body was swabed or secured (100%; 1/1) and the 4 alerts were investigated within 24 hours (100%; 4/4).

Table 5. Status of key PoE/PoC indicators by province, as of November 6, 2026

Indicators	Ituri	North-Kivu	South Kivu	Tshopo	Bas-Uele	Overall
Report completeness	92% (57/62)	95% (20/21)	56% (5/9)	100% (18/18)	50% (11/22)	84.1% (111/132)
Strategic PoC/PoE	1	N / A	N / A	N / A	N / A	1
Number of past travelers	147 103 95	793 7 173		23,989	2,345	276,403
Proportion of travelers screened	97%	99%	98%	100%	98%	98%
% of travelers who washed their hands	96%	99%	98%	99%	98%	97%
% of those travelers made aware	100%	99%	100%	100%	76%	99%

Machine Translated by Google

Number of alerts notified	02	02	00	00	00	4
Number of validated live alerts	02	01	00	00	00	3
Number of lifeless bodies intercepted	00	01	00	00	00	1
% of suspected cases transferred to CT/CTE	100% (2/2)	100% (1/1)	0%	0%	0%	100% (3/3)
% of lifeless bodies swabed/secured	0%	100% (1/1)	0%	0%	0%	100% (1/1)
% of suspected cases investigated within 24 hours	100% (2/2)	100% (2/2)	0%	0%	0%	100% (4/4)

Laboratory

2.2.2. Main actions

- **Ituri:** 24 new positive results (14 living and 10 dead) out of 279 samples received and analyzed (positivity: 8.6%).

North **Kivu:** 12 new confirmed cases (6 living and 6 dead); laboratories in Beni and Butembo have 279 samples were received and 276 were analyzed (positivity: 4.3%)

- **Tshopo :** 0 new positive results out of 10 samples received and analyzed, all negative
- **Haut-Uélé :** 5 new positive results (4 living and 1 death) out of 19 samples analyzed in the laboratory d'Isiro (positivity: 26.3%).

.

2.3. Infection Prevention and Control (IPC/EDS)

2.3.1. Main actions

North **Kivu :** PCI-WASH activities are still showing insufficient results, with only 5 out of 6 rings open and a significant gap in the Beni health zone. Regarding decontamination, only 4 out of 17 identified households were decontaminated in Katwa, Butembo, and Beni, while 6 out of 10 identified health establishments were decontaminated in Kalunguta, Butembo, Beni, and Masereka. Furthermore, none of the 4 decontaminated households received any supplies, revealing a major gap in all the health zones concerned. The 11 PCI-WASH Scorecard assessments conducted in 11 health establishments showed an average score of 32.2%, reflecting a low level of compliance with PCI-WASH standards. In addition, the risk assessment of staff exposure revealed 2 out of 5 high-risk cases in Musienene. Finally, one case of infection among frontline workers was recorded among the 24 confirmed cases in the Katwa health zone. These results underscore the need to strengthen IPC-WASH interventions, accelerate decontamination activities, ensure the provision of supplies to affected households, and reinforce protection measures for frontline workers.

- **Tshopo:** operational monitoring of the Ebola response continued in the health zone, assessment of transit sites, briefing of supervisors at the DPS and on-the-ground support of 23 providers trained in standard precautions in two health facilities.

2.3.2. **Haut-Uélé:** low completeness of reports: 4 out of 7 health zones (57.1%) sent their data; lack of PCI kits for ESS; activation of EDS teams trained by FHI 360 in the different health zones (Isiro)

2.4. Holistic care

2.4.1. Main actions

- **In Ituri,** 79 new admissions (all suspected, including 2 in advanced stage) were recorded against 71 discharges, including 3 recoveries and 3 discharges without medical advice in Nizi and Biakato. Thus, 363 patients (130 confirmed and 228 suspected) remain in isolation at the end of the day (363 the day before) (To the team: 363 the day before + 79 admissions ÿ 71 discharges = 371 and not 363; 130 confirmed + 228 suspected = 358; provide the exact numbers), representing an overall occupancy rate of 36%; 233 out of 363 patients (64.2%) received hot meals, all children under LNPE/SLM were taken care of (8/8 at the CTE, 4/4 in daycare and 66/66 in the community), 31 patients were evacuated and 18 out of 34 transfers (52.9%) were organised.

Machine Translated by Google

In North Kivu, 81 new admissions (6 confirmed and 73 suspected) were recorded, compared to 99 discharges, including 1 recovery. A total of 554 patients (191 confirmed and 363 suspected, including 71 children under 5 and 24 critically ill) remained in isolation at the end of the day.

Of the 24 most recent confirmed cases, 17 (70.8%) (Note to the team: this paragraph pertains to the SMSPS; North Kivu had 12 new cases on November 7, not 24; please move it to the SMSPS section and update it.) are currently being monitored and treated, while 223 confirmed cases are also being monitored. 277 new suspected cases were recorded, of which 265 were treated and are being monitored, with 104 test results announced, 52 of which were positive. Furthermore, 549 people, including 183 men, 267 women, and 99 children, benefited from psychoeducational sessions with UNICEF support, while 103 households received public health support and 78 people living with HIV/AIDS (PLHIV) received assistance during the day. Finally, all 11 expected recoveries were followed up in the community, representing 100% coverage.

2.4.2. In Tshopo, of the 667 people who received experimental treatment, 601 benefited from Post-Exposure Prophylaxis (of which 473 completed it, 63 are ongoing and 65 have discontinued it) and 66 confirmed cases were treated — mostly with Remdesivir (63) and marginally with MBP134 (3) —
recording 44 recoveries, 16 deaths, 5 ongoing treatments and 1 escape.

In Haut **-Uélé,** 13 new admissions (3 confirmed in Pawa, 1 in Wamba and 9 suspected) were recorded.

2.5. Vaccination

2.5.1. Main actions

- **North Kivu**

A total of 17 breastfeeding women were registered (To the team: no daily or cumulative vaccination counts; provide daily vaccinations, cumulative totals, and doses by province), with no reported cases of pregnancy, while 7 individuals from special populations (displaced persons/refugees) were identified. All vaccinated individuals were 18 years of age or older. Furthermore, slightly more than half of those vaccinated reported having previously received an Ebola virus disease (EVD) vaccination.

- **In Tshopo, monitoring of the two waves of Ebola vaccination in the DPS shows a decline**
progressive completion of reporting in the six health zones over the course of the days. Meanwhile, the daily pace of vaccinations indicates generally higher coverage during the second wave (V2) than the first (V1), although daily numbers rarely exceed the target of 400 people per day.

2.6. Continuity of Care

2.6.1. Main actions • In North Kivu,

The baseline assessment of Musienene General Referral Hospital reveals major deficiencies in the continuity of essential care, including the absence of triage and emergency services, insufficient personal protective equipment (PPE), and weaknesses in maternity, pediatric, and nutrition services. Despite the absence of any specific issues reported in neonatology, gaps also persist in patient follow-up, the availability of human resources, and the quality of services, necessitating an urgent strengthening of the facility's capacity and equipment.

- **In Tshopo, the main activities** focus on strengthening governance and skills through the coordination of the Integrated Management System (IMS), the pillar meeting, and transfusion safety training (Day 4). They also encompass the operational management and evaluation of the healthcare network, including the audit and remediation of 5 Integrated Health Establishments (IHEs), the analysis of indicators on DHIS2, and the monitoring of providers at the Transfusion Center (TC) and Transfusion Treatment Center (TTC).

Machine Translated by Google

2.7. Risk Communication and Community Engagement (RCCE)
2.7.1. Main actions

• **In North Kivu, community awareness activities** focused on Ebola prevention, reporting of alerts, referral of patients to health facilities, handwashing and the importance of referral to the Ebola Treatment Center through home visits and educational talks in several health zones.

In addition, mass communication actions and briefings with local leaders have been strengthened in order to combat rumors, promote community engagement, foster acceptance of public health activities and mobilize communities around compliance with Ebola prevention measures.

In Tshopo, activities focused on risk communication and community engagement (RCCE) briefed 50 members of women's associations and leaders of sports clubs in Mangobo to counter resistance, promote vaccination, and encourage early management of Ebola virus disease (EVD). These activities also supported the coordination of the response (zonal Integrated Management System and community surveillance) while bolstering medical care by raising awareness among seven healthcare providers regarding prophylaxis and contact tracing in the field.

.

2.8. Mental Health and Psychosocial Support (MHPSS)
2.8.1. Main actions

In North **Kivu, mental health and psychosocial support activities** enabled the monitoring of 17 out of 24 confirmed cases (70.8%), the support of 28 out of 31 separated and orphaned children (90.3%), and the care of all 6 children in daycare (100%), with 16 recovered and reintegrated. Furthermore, 28 out of 36 children (77.8%) received community-based follow-up, while psychoeducational sessions, household support, and psychosocial support continued to strengthen the care provided to those affected by the epidemic.

2.9. Logistics
2.9.1. Main actions

• **In North Kivu, the delivery** of EDS and IPC inputs was carried out in the Beni health zone for the launch of the activities of the ECUMERS IPC inputs (body bags, gloves, garbage bags, thermoflash, safety boxes, gowns, plastic boots and gel) were delivered to the Kalunguta health zone, while a shipment of IPC inputs destined for Goma on behalf of the WHO is currently in transit in Beni.

• **In Tshopo, the interventions focused on finalizing** the setup and relocating the team
Post-Exposure Prophylaxis (PEP) to the Cinquantenaire Hospital, the transfer of heavy equipment from the isolation site by crane and the field assessments carried out by the WHO in the ESS for triage and the estimation of work.

2.10. Security
2.10.1. Main actions

• **In Ituri,** the Avakubi PoC remains non-functional due to insecurity; in North Kivu, a grenade was discovered in front of the entrance to the Beni Health Zone office ; EDS teams were accompanied and secured for the burial of two confirmed cases at the Bundji cemetery and for swabs in Butuhe and the Vighole neighborhood; the IPC team was escorted for the decontamination of a household in Manziko (Benengule health district); and the threat against PoC providers on the Butembo axis persists; **in Haut-Uélé,** the security threat against PoC providers persists on the Bolebole axis (Pawa and Wamba); in Tshopo, the house of a RECO was burned down in Bafwasende, where response activities remain suspended, and young people attempted to recover a lifeless body; In Bas-Uélé, the pillar assessed the security situation, secured the response and monitored activities at the zonal level.

2.11. PSEA (Prevention of Sexual Exploitation and Abuse)
2.11.1. Main actions

In Ituri, 2 members of the INRB CREC team (1 woman and 1 man) were briefed on the PSEAH and signed the code of good conduct, 22 wood sellers (12 women, 7 men and 3 boys) were sensitized in the Bunia health zone and the display of messages and the distribution of leaflets continue in the health zones of Bunia, Aru, Mongbwalu, Mambasa, Fataki and Mandima;

In North Kivu, in the Mutwanga health zone, 13 members of the Kasindi-Lubiriha isolation center team, including 7 women and 6 men, received a briefing on the prevention of sexual exploitation and abuse (PSEA). Furthermore, in the Kalunguta health zone, a community awareness session on PSEA was organized at the Mataba health area's community health center for 15 participants, including 13 women and 2 men, primarily pregnant women.

Machine Translated by Google

In Haut-Uélé, 9 PPLs from the Anualité hospital in Matali (5 men and 4 women, Wamba health zone) were sensitized on the PSEA, referral mechanisms and the rights of victims; in Bas-Uélé, 23 learners from the ITM in Dingila (17 boys and 6 girls) were sensitized on the PRSEAH.

2.11.2. Challenges

- Coverage of the affected areas remains limited: only Ituri, North Kivu, Haut-Uélé and Bas-Uélé have reported their PSEA activities for the day (4 out of 8 DPS; 50.0%); in Bas-Uélé, 4 ZS (Viadana, Bondo, Likati and Ango) remain silent, funding for the pillar is low and posters and banners are out of stock; the reporting mechanism is known in only 15 out of 36 ZS (41.7%) in Ituri and 6 out of 13 ZS (46.2%) in Haut-Uélé, only one partner (WHO) reports its PSEA activities in Haut-Uélé, some partners refuse to align in Ituri and awareness tools in French and Lingala, visibility materials and computer kits for the pillar remain insufficient.

KEY MESSAGES

Forty-one (41) new confirmed cases were reported in three provinces, including 24 in Ituri, 12 in North Kivu and 5 in Haut-Uélé. transmission remains particularly marked in the Beni ZS (6 cases) and Mabalako (3 cases) in North Kivu and in those of Mandima (8 cases) and Nizi (4 cases) in Ituri, as well as Pawa (4 cases) in Haut-Uélé.

Of the 21 confirmed deaths reported, 12 are deaths community-based (5 in Ituri, 6 in North Kivu and 1 in Haut-Uélé) and 9 are deaths at CTEs (5 in Ituri, 3 in North Kivu and 1 in Haut-Uélé). The contact tracing rate is 69.8% (23,276/33 346) in the five provinces with contacts under follow-up (Bas-Uélé: 99.6%; Ituri: 82.9%; Haut-Uélé: 80.9%; North Kivu: 62.3%; Tshopo: 49.3%), a slight decrease compared to the previous day (71.7%) (To the team: Q3) and still below the 95% threshold, with the lowest rate in Tshopo, where the data from Bafwasende does not are not available due to the cessation of activities for cause of insecurity.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic is moving beyond the detection of the first cases suspected of having the virus confirmed by biological testing Ebola Bundibugyo, leading to the declaration official of the 17th epidemic of Virus Disease Ebola in the Democratic Republic of Congo.

Between May 17 and 18, 2026, the authorities national, with the support of partners, have strengthened the response with the declaration of the public health emergency of scope national and then continental, accompanied by

Machine Translated by Google

EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP
Dr. MWAMBA KAZADI Dieudonné

Email:diedonne.mwamba@insp.cd
Incident Manager
Professor Dr. AHUKA MUNDEKE Steve
Email: steve.ahuka@unikin.ac.cd

Operations Manager
Dr. KOMBE NGWAMA John

Editorial Team
Professor Dr. MULANGU KATULUMBA Félix-Médard
Dr. Jules Katusi Biselenge

Contact:
www.insp.cd
contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd